Patient Group Direction: **Adrenaline 1:1000 Injection (PFS / ampoule)**			
Version: **3.0**	Date of introduction: **21/10/2024**	▼Black Triangle Product: **NO**	Classification: **POM**

For the administration only of the above medicine(s) by Registered Practitioners employed by the Trust.

This patient group direction must be agreed to and signed by registered health care professionals involved in its use. The service lead or manager is responsible for holding the original signed copy in the clinical setting where the PGD will be used. An electronic copy can be found in the approved PGD database on the Trust intranet.

This document is not valid if printed
For reference only

Developed / Reviewed by:		
Lead Author	**Name**: Katy Stevens **Signature**:	**Position**: Renal Pharmacist **Date**: 21/06/2024
Physician/Consultant*	**Name**: Dr Helen Clarke **Signature**: Helen Clarke	**Position**: Consultant nephrologist **Date**: 21/06/2024
Lead Nurse / Allied Health Professional/ Lead Practitioner	**Name**: Heather Atkins **Signature**: H.Atkins	**Position**: Senior matron, Renal Services **Date**: 21/06/2024
Pharmacist*	**Name**: Katy Stevens **Signature**:	**Position**: Renal Pharmacist **Date**: 21/06/2024
ALL signature boxes above MUST be complete before submission to Medicine Management Group for approval.		
Organisational authorisation by MMG (Chair, or Deputy)	**Name**: Joe Maguire **Signature**:	**Position**: Deputy Chief Pharmacist **Date**: 21/10/2024 **PGD Expiry Date: Jun 2027** (no longer than 3 years)

*For PGDs covering systemic antibiotic use, a microbiologist and/or antimicrobial pharmacist must be included in its development.

Version	Summary of Changes
3.0 (Replaces 2.0)	New template with version numbers. Nil other material change

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You must be authorised by name under the current version of this PGD before you attempt to work according to it.

1. Clinical Condition

Indication	Emergency treatment of acute anaphylaxis in patients treated by Renal Anaemia Service and in patients receiving treatment at Renal Satellite Dialysis Units SEND A RESPONSIBLE ADULT TO CALL 999 AND STATE THERE IS A CASE OF SUSPECTED ANAPHYLAXIS Tick this box if an 'off-label' use of the drug: ☐
Inclusion criteria	Patients meeting the criteria as defined in the Resuscitation Council / Trust policy for management of anaphylaxis Anaphylaxis is likely when all of the following three criteria are met: • Sudden onset and rapid progression of symptoms • Life-threatening airway and/or breathing and/or circulation problems • Skin and/or mucosal changes (flushing, urticarial, angioedema) Remember: • Skin or mucosal changes alone are NOT a sign of an anaphylactic reaction • Skin and mucosal changes can be subtle or absent in up to 20% of reactions (some patients only have a decrease in blood pressure i.e. a circulation problem) • There can also be gastrointestinal symptoms (e.g. vomiting, abdominal pain, incontinence) Most reactions will occur over several minutes but rarely may be slower in onset. The patient is usually
Exclusion criteria	Contraindications: none
Cautions / Need for further advice	In severe, genuine anaphylaxis there are no relevant cautions or contraindications Anaphylactic reaction should be distinguished from syncope and panic attacks.
Action if excluded	Discuss URGENTLY with a doctor if possible document 'Informed refusal' patient notes. Inform relevant healthcare professionals if patient is transferred for further management.
Action if patient declines treatment/refuses medication	As per if excluded

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2. Description of Treatment

Method or route of administration	Adrenaline 1:1000 (1mg/1mL) injection as prefilled syringe (use 0.5mg / 0.5mL per dose) **or** Adrenaline 1:1000 (1mg/mL) ampoule (use 0.5mg / 0.5mL per dose) May be administered as an intramuscular (IM) injection preferably to the outer thigh or upper arm
Dose	Adults and children over 12 years: 0.5mg – 0.5mL As intramuscular injection (IM)
Frequency	Single dose
Duration of treatment	A second dose may be given after 5 minutes if there is no clinical improvement. Maximum TWO doses only.
Quantity to supply / administer	Administration ONLY. • 0.5ml of 1:1000 (1mg/1ml) ampoule or prefilled syringe (PFS) - discard remainder. • Use a second ampoule or PFS if a further dose is necessary.
Storage	Ampoules and PFS: • Keep container in the outer carton. • Do not store above 25°C. • Do not freeze.

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3. Patient Information

Advice and information to patient/carer, including side effects	• Unusual or life-threatening reactions require immediate medical attention.

- Side effects include: palpitations, tachycardia, sweating, nausea, vomiting, dyspnoea, anxiety, tremor and cold extremities.
- Before discharge, a healthcare professional with the appropriate skills and competencies should offer patients (or, as appropriate, their parent and/or carer) the following:
  - Explain current course of action & need for urgent medical attention.
  - Information about anaphylaxis, including the signs and symptoms of an anaphylactic reaction.
  - Be aware that up to 20% of people with anaphylaxis have biphasic (two-phase) or prolonged anaphylaxis, i.e. a reaction that resolves and then recurs later without further exposure to the trigger. The late phase reaction usually occurs within eight hours, but may occur up to 72 hours after the initial symptoms.
  - A person with protracted anaphylaxis has signs and symptoms that persist for hours or even days despite treatment, although this is rare. Patients experiencing biphasic anaphylaxis should contact emergency services without delay.
- Advice about how to avoid the suspected trigger (if known).

Follow-up advice to be given to patient or carer

- All patients must be referred for urgent hospital treatment even if they appear to have made a full recovery.
- Allergies and suspected adverse drug reactions should be clearly documented in the patient's notes and the GP/other relevant healthcare professionals informed.
- Any serious adverse drug reaction should be reported to the MHRA using the Yellow Card Scheme (www.mhra.gov.uk)

4. Staff Characteristics

Registered Profession

(tick those that apply)

- ☐ Chiropodists & Podiatrists
- ☐ Dental hygienists
- ☐ Dental therapists
- ☐ Dieticians
- ☐ Midwives
- ☑ Nurses*
- ☐ Occupational Therapists
- ☐ Optometrists
- ☐ Orthoptists
- ☐ Orthotists & Prosthetists
- ☐ Paramedics
- ☐ Pharmacists
- ☐ Physiotherapists
- ☐ Radiographers
- ☐ Speech & Language Therapists

*Nursing Associates not eligible to use PGDs

Additional Qualifications

Nil in addition

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Specialist Competencies	• Completion of Trust mandatory Anaphylaxis training • Completion of Immediate Life Support course due every 3 years • Attendance at annual Renal Update Day
Additional Requirements	• Evidence of CPD, and any training/competences relevant to the PGD. • Working knowledge of organisational policies (including Medicines Management Policy), Anaphylaxis Guidance (resus.org.uk) & Safeguarding Policies and related mandatory training.

5. Record keeping, Referral Arrangements and Audit Trail

Specify method of recording supply /administration including audit trail	• Document allergies and other adverse drug reactions clearly in patient records and inform the GP and other relevant practitioners/patient/carer for further reporting and action if required. • Report any adverse drug reactions to the (MHRA) through the yellow card reporting system (www.mhra.gov.uk). The following will be recorded in the patient's health records: • The dose administered and/or the quantity supplied.
6. References used in the development of this PGD:	• BNF (British National Formulary) online accessed – 14 / 05 / 2024 • Summary of Product Characteristics 14/05/2024 – 2024 (PFS) & 3673 (amps) • Working group of UK Resuscitation Council UK, Guidelines for the Emergency Treatment Anaphylactic Reactions, 2021.

Contacts for more information on the status of this document	
Royal Devon PGDs Pharmacy Administrative Office Pharmacy Department RD&E Hospital (Wonford) Tel: 01392 402441	
PGD Template Version: ***2022-1***	*Template approved by MMG Chair (April 2022)*

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PGD's DO NOT REMOVE INHERENT PROFESSIONAL OBLIGATIONS OR ACCOUNTABILITY

It is the responsibility of each professional to practice only within the bounds of their own competence and in accordance with their own Code of Professional Conduct.

Note to authorising Managers: A paper copy of the full PGD must be kept in the relevant clinical area together with this sign-off sheet. Authorised staff can be signposted to the intranet where they can view the document.

CLINICAL AREA	LOCATION / DEPARTMENT

I have read and understood the Patient Group Direction and agree to supply / administer this medicine only in accordance with this PGD.

Name of Registered Professional	Signature	Authorising Manager	Date
			DD / MM / YYYY
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